

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 73 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: March 17, 2018 | Provider: Raymond Foster, MD — Cardiology | Doc ID: SMC-2018-0317-DC

Hospital Discharge Summary

Acute Decompensated Heart Failure — HFrEF (EF 35%)

Admission Summary

Admitted / Discharged March 11, 2018 / March 17, 2018 (6 days)
Attending Raymond Foster, MD (Cardiology)
Discharge Disposition Home with cardiac home-health and HF clinic follow-up
Discharge Weight 94.2 kg (net –4.8 kg from admission)

Hospital Course

Diuresed with IV furosemide to euvolemia over 5 days (net –4.8 kg); dyspnea and orthopnea resolved, oxygen saturation normalized on room air. Rate controlled; remained in AF at ~70-80 bpm. Guideline-directed medical therapy initiated and up-titrated as renal function and blood pressure permitted. Creatinine peaked 1.7, improved to 1.5 (eGFR 47) at discharge. Potassium stable. HF education and low-sodium diet (<2 g/day), fluid restriction (1.5 L/day), and daily weight monitoring taught.

Discharge Diagnoses

1. Acute decompensated heart failure with reduced ejection fraction (EF 35%), new.
2. Atrial fibrillation, rate-controlled, anticoagulated.
3. Chronic kidney disease stage 3b (cardiorenal).
4. CAD s/p DES; T2DM; treated prostate cancer (stable); dyslipidemia; mild anemia of chronic disease.

Discharge Medications (Reconciled)

Medication	Dose	Route	Frequency	Indication
Sacubitril/valsartan	24/26 mg	PO	Twice daily	HFrEF (replaces lisinopril)
Carvedilol	6.25 mg	PO	Twice daily	HFrEF (replaces metoprolol)
Spironolactone	25 mg	PO	Once daily	HFrEF (MRA)
Furosemide	40 mg	PO	Once daily	Congestion
Empagliflozin	25 mg	PO	Once daily	HFrEF + T2DM
Apixaban	5 mg	PO	Twice daily	AF anticoagulation
Atorvastatin	80 mg	PO	Nightly	ASCVD
Metformin	1000 mg	PO	Twice daily	T2DM (hold if acutely ill)
Tamsulosin	0.4 mg	PO	Nightly	BPH

Follow-up & Instructions

- HF clinic in 7 days; cardiology in 2-4 weeks for GDMT up-titration.
- Labs (BMP) in 5-7 days to monitor renal function/potassium after ARNI + MRA initiation — note ACEi (lisinopril) stopped; 36-hour washout observed before starting sacubitril/valsartan.
- Lisinopril DISCONTINUED. Aspirin remains off (on apixaban).
- Return for weight gain >2 kg in 3 days, worsening dyspnea, or chest pain.